Beyond "Why Me?"

Music Therapy and Emotional Knowing in the Face of Finitude

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1. The Collapse of Categories

While medical systems rely on categorical distinctions to organise care, from the perspective of lived mortality, these distinctions eventually cease to exist. Whether in an oncology ward or a palliative unit, patients are confronted with the same existential rupture, the sudden exposure to mortality that gives rise to the question, *"Why me?"*.

The more I sit with this question, the more it multiplies. The water muddies; certainty recedes. Medical care can often provide explanations, diagnoses, probabilities, and treatment pathways, but these answers rarely address what is being asked. Sometimes they do the opposite: they confirm that the wound is not informational. Even if a complete explanation were possible, it would not, by itself, dissolve the existential and psychological pain that has already taken hold. And to be fair, this is not only a limit of Medicine or science. Even if philosophy, psychology or theology could offer a perfectly sufficient causal account, whether in the language of karma, divine will, fate, or moral order, the wound would not necessarily close. What remains is not a demand for more explanation, but for a way of bearing and knowing what cannot be solved.

This moment suggests a different kind of orientation. The crisis is not simply a lack of explanation, nor can it be resolved by expanding causal accounts. What is at stake is not knowing why in the explanatory sense, but knowing how to remain—how to stay present to one's life when its continuity has been threatened. This form of knowing is neither propositional nor explanatory. It does not resolve uncertainty, but reorganises what can be felt, endured, and shared.

In clinical encounters with existential suffering, such "knowing" often emerges not through interpretation or reassurance, but through embodied and relational practices that make the unbearable bearable without pretending it is solved. These practices do not negate finitude, nor do they promise reconciliation. Instead, they alter the conditions under which suffering is lived, shifting attention, affect, and relational presence in ways that allow experience to be held rather than escaped.

2. What Kind of Question Is *"Why Me?"*

The paradox of the question *"Why me?"* lies in the fact that it is asked despite an implicit knowledge that no answer will suffice. And yet, more often than not, it is treated as a request for explanation. It is therefore met with statistics, risk factors, genetic predispositions, lifestyle histories, or probabilistic models. In my daily clinical encounters, however, it quickly becomes apparent that the question persists even when such explanations are offered, and sometimes precisely because they are offered.

Patients may consciously seek reasons, fairness, and certainty. But on a deeper level, the question often expresses shock, injustice, fear, loss of control, and the felt impossibility that "this should not happen to me." Many recognise, sometimes even articulate, that no explanation, however complete, will undo what has occurred. And yet ironically enough, the question cannot be avoided. It arises insistently, not as a rational inquiry, but as an attempt to re-establish orientation when the continuity of one's life has been broken. In this sense, the question is structurally futile yet existentially unavoidable: it reaches beyond what can be answered because the human subject cannot remain silent in the face of radical contingency. This is not confusion, strictly speaking, but a human contradiction under finitude.

This suggests that *"Why me?"* is not primarily a causal question. It does not seek a chain of events that led from one state of affairs to another. Rather, it arises when a life is interrupted, when the continuity that once made the future feel implicit is abruptly withdrawn. In this sense, the question is less about origin than orientation: not "What caused this?" but "How am I to live now that this has happened?"

The persistence of the question, despite explanatory answers, reveals a category mismatch. Explanations address events; existential questions address persons. To answer the latter with the former is not only insufficient but also alienating, as if the question has been misunderstood at its core. What is being asked is not for information, but for a way of inhabiting a world that has suddenly become unfamiliar.

What makes this confrontation so devastating is precisely the absence of anyone to blame. Some of the patients express regret that they had lived in a constant seriousness, as if that seriousness now intensified the injustice of what was happening.. The suffering feels unjust, yet there is no perpetrator, no moral failure, no corrective action that could set things right. Blame may be cast on fate, on others, on God, on the body itself, but it does not resolve the rupture. It is just another temporary grapple with reality, an avoidance of confronting the insoluble. The injustice remains, and with it the question that cannot be answered or abandoned. The question repeats not because it is poorly formed, but because it seeks an orientation that no objective account can deliver.

Recognising this distinction does not diminish the importance of medical explanation. Rather, it clarifies its limits. Explanatory knowledge remains indispensable for treatment and decision-making, but it cannot, on its own, reorient a person whose relationship to time, body, and future has been disrupted. The demand expressed in *"Why me?"* points toward another register of knowing, one concerned less with explanation than with presence, endurance, and meaning.

In this sense, the existential encounter with serious illness is not merely distressing or frightening. It is a confrontation with a form of suffering for which there is no culprit and no appeal, a condition that many patients describe, without exaggeration, as hell.

3. When Explanation Reaches Its Limit: Toward Another Kind of Knowing

If the question *"Why me?"* persists because it cannot be answered, then what comes into view is not merely a limit of explanation, but the loss of orientation itself. Explanatory knowledge—whether medical, philosophical, or theological aims to make events intelligible by situating them within causal, logical, or moral frameworks. In doing so, it answers questions of origin, mechanism, and justification. Yet the clinical scenes described above suggest that intelligibility alone does not restore a person's footing when a life has been disrupted at its core.

This does not mean that the explanation is worthless. It remains indispensable for treatment, decision-making, and care planning. But existential rupture exposes a situation in which even the best explanations may coexist with total disorientation. In such moments, the problem is not primarily ignorance, nor is it necessarily solvable by "more understanding." In clinical practice, one sometimes witnesses profound disorientation after serious prognostic news. In such moments, the world can become unrecognisable even when it has been fully described.

It is tempting at this point to claim that another kind of knowing "takes over", that one can learn how to remain, how to stay present to life when its continuity has been fractured. But lived experience does not grant such guarantees. There are moments in which no orientation is available: no insight, no acceptance, no narrative coherence, and no inner stance strong enough to quiet the shock. Sometimes the most truthful description is not "knowing how to remain," but simply enduring without any stable way to inhabit what is happening.

Such shifts are not guaranteed, but they are familiar in clinical work. I have often seen people continue to request music even after difficult news, and even when their physical capacity is significantly limited. These gestures—seeking air, movement, familiar surroundings, or sound—can persist as small movements toward life, even when the future has narrowed.

When such shifts occur, they are often neither propositional nor explanatory. They show themselves instead as changes in bodily and temporal organisation: breathing that returns, attention that steadies, the possibility of staying with fear without tipping into collapse. This is not knowledge in the sense of conclusions. It is knowledge as a reconfiguration of presence, an altered capacity to endure, to share, to remain in relation.

Importantly, this register of knowing is not private in the sense of being inaccessible or merely subjective. It emerges in interactions and can be witnessed in clinical practice: when panic becomes tolerable, when despair becomes speakable, when silence becomes holdable, when a person regains a minimal sense of contact with their own life. Such moments are not the by-products of explanation. They are practical achievements, fragile, contingent, and often temporary, that make continuing possible when no answer can.

To speak of *emotional knowing* here is not to psychologize suffering or reduce it to inner states. It is to recognise that emotions participate in how reality is encountered. They disclose what matters, shape what is bearable, and determine what can be approached or must be avoided. In this sense, **emotions function epistemically:** they are part of how a person comes to know their situation and their possibilities within it, even when those possibilities are reduced to the smallest margins.

What emerges at the limit of explanation, then, is not irrationality and not necessarily resignation. It is the exposure to a different task. Sometimes there is no way to remain. Where there is, it is not produced by argument alone. It is produced through bodily, relational, and temporal practices that reorganise experience enough to allow a person to live with what cannot be solved.

4. Music as a Mode of Orientation

So many times I have heard patients say, often in an attempt to confront their situation with a kind of stoic clarity—"Everyone has to die," "For everyone it ends sooner or later," or "I've lived my life; I have no regrets." Acceptance and resilience are terms often invoked in clinical psychology, and I do not doubt their dignity. But I have often sensed how fragile they are: more like a foothold than an arrival, more like a posture adopted against panic than a settled reconciliation.

The difficulty is not that acceptance or resilience are "wrong," but that their object remains unclear. Accepting what, exactly—death? contingency? the loss of a future? And resilient in what sense—enduring treatment, enduring uncertainty, enduring decline? Many things in life can be endured and integrated into a narrative. Death cannot be integrated in the same way, because it is not one event among others; it is the limit of all events. This is why, in the face of mortality, even courageous sentences can fail to hold.

In many cases, after such proclamations, only a few notes are needed before tears begin to flow and do not stop. Something in the body contradicts what the mind tries to declare. This is not a failure of character. It is a sign that existential rupture is not resolved by sentences, no matter how brave those sentences may sound.

And this is where music becomes philosophically and clinically significant. "Everything has to end"—yes. A musical piece ends as well. And yet we still listen. We return to it. We ask for it again. The question is not whether finitude can be named, but what it means to remain present to finitude without reducing oneself to irrelevance—and why music so often becomes one of the few ways in which that remaining is possible. If so, one might ask: what kind of orientation is music offering when words, brave as they may be, fail to hold?

Kierkegaard writes, "The self is a relation that relates itself to itself[1]." Whatever one makes of his theological conclusion, the phenomenological insight is striking: being human is not simply existing, but having to hold oneself together across tension—between finitude and what exceeds it, between the body's limits and the mind's reach. In moments of despair, it is not always that meaning is missing, but the capacity to maintain this relation without collapse.

In the history of art and illness, the Isenheim Altarpiece[2] is often invoked as an example of empathic mirroring[3]: an image placed in a hospital context in which the suffering body of Christ is rendered with striking physical realism. Whatever its precise historical functions, it offers a compelling model of how art can accompany pain without explaining it away. The

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[1] Søren Kierkegaard, The Sickness unto Death, trans. Alastair Hannay (London: Penguin, 1989), 43.

[2] The Isenheim Altarpiece (Matthias Grünewald, 1512–1516).

[3] *Christ's suffering body was intended to mirror the patients' physical afflictions, enabling identification rather than distance.* Andrée Hayum, The Isenheim Altarpiece: God's Medicine and the Painter's Vision (Princeton, NJ: Princeton University Press, 1989). 71–96.

image does not alleviate the viewer's suffering; it makes suffering visible, shareable, and holdable within a symbolic form. In that sense, it creates a space where the unanswerable can be endured—if not transformed, then at least met without abandonment.

In clinical work, this fragility of the self as relation is often visible before it is fully speakable.

In composite clinical observations, difficult news may be followed by abrupt bodily disorientation, including changes in breathing, narrowed attention, and a diminished sense of temporal continuity. At times, quiet music-making can help sustain contact long enough for a minimal steadiness to return. Nothing is resolved, yet a minimal capacity to remain can re-emerge.

At other times, the rupture returns through repetition—fear resurfacing in a recurring image that feels less interpretable than inevitable. In composite form, some patients describe recurring distressing dreams or images through which fear seems to return in repetitive form. The point is not a lack of explanation, but the absence of a place to stand: whether the self can remain intact in relation—without mastery and without resolution—when the circle tightens again.

If the self is a relation, then music offers a privileged medium of orientation. Music exists only as a relation in time: it binds breath, pulse, limitation, and attention, while also opening resonance, significance, and emotional depth—without requiring that these tensions be resolved. In clinical music therapy, despair does not necessarily require explanation; sometimes it requires containment. Through shared rhythm, pulse, and improvisation, the self can be held together by sound long enough to persist where discursive meaning fails.

Music offers orientation not by explaining finitude, but by placing the listener within it. Unlike verbal assertions of acceptance, which operate at the level of meaning and belief, music works through time, embodiment, and relation. It unfolds moment by moment, requiring no commitment to conclusions and no declaration of stance. One does not need to decide what one thinks about death in order to remain with a musical phrase; one only needs to stay. In this way, music mirrors finitude without negating it: it begins, it unfolds, it ends—and yet the ending does not retroactively empty what preceded it. The piece does not justify itself, but neither does it collapse into irrelevance because it ends.

This is why music can accompany existential suffering without demanding reconciliation. It does not ask the patient to “mean” something, to accept a doctrine, or to narrate coherence where none is felt. Instead, it offers a lived structure in which time can be inhabited again—sometimes only briefly, sometimes tenuously, but often enough to allow breathing to lengthen, attention to settle, and presence to be sustained. The knowledge enacted here is not the knowledge of propositions, but of participation: a knowing-with rather than a knowing-about, in which the self is not explained but held in relation to what cannot be explained away.

If music can function in this way—reorganising presence where explanation cannot—then the clinical question is not only whether it “helps,” but what exactly is being done, by whom, and under which conditions. What changes in time, attention, breath, and relational contact when certain musical choices are made? Which elements of the practice are essential, which are incidental, and when does it fail? These are not questions that can be answered by outcomes alone, because the phenomenon is not merely the reduction of symptoms but the

reconfiguration of lived experience. This is where an artistic-phenomenological approach becomes methodologically necessary: it can describe the intervention from within its own medium (sound, timing, attunement) while remaining accountable to observable shifts in clinical reality.

5. From Experience to Inquiry: Emotional Epistemology and Artistic Research as Method

If music can function in the ways described above—reorganising presence where explanation cannot, then the task of research is not to translate this experience into abstract theory, but to inquire into how such reorganisation occurs, under what conditions, and with what limits. The difficulty is methodological. Conventional clinical research excels at measuring outcomes, symptom reduction, and behavioural change over time. It is far less equipped to describe temporary, embodied, relational shifts in lived experience that are yet decisive in whether a moment is endured or becomes overwhelming.

What is at stake here is a different understanding of knowledge itself. Emotional epistemology starts from the premise that emotions are not merely subjective states, internal moods, or by-products of cognition, but ways in which reality is disclosed and navigated. Emotions shape what becomes salient, what draws attention, what feels threatening or tolerable, and what kinds of action are possible in a given moment. In existentially charged contexts, emotions are not obstacles to understanding; they are part of how understanding takes place.

This premise is supported by contemporary theories of emotion that emphasise their embodied and action-oriented character. Emotions are not detachable inner experiences that occur after bodily reactions; they involve bodily sensations and dispositions to move and respond. As Deonna and Teroni argue, emotions can be understood as *felt bodily attitudes of action-readiness* toward significant situations or events[4]. Because of this structure, emotional episodes directly influence perception, attention, and immediate behaviour, often prior to reflective judgment. As Slaby has shown, emotions are not merely felt *in* the body but enacted *through* it, organising how the world appears and how one is able to respond[5].

In clinical contexts of serious illness, this action-guiding dimension of emotion becomes especially pronounced. Fear, despair, or uncertainty are not merely thoughts to be corrected; they are bodily states that tighten breath, narrow attention, disrupt temporal continuity, and constrain the ability to remain present. When patients describe feeling overwhelmed, frozen, or unable to stay with what is happening, they are not primarily reporting faulty beliefs. They are describing a breakdown in emotional orientation.

In the Healthcare environment, artistic research[6] may offer a methodological framework for engaging this dimension, but only under specific conditions. In what follows, I use the term in a narrower sense: as inquiry grounded in situated artistic practice, accompanied by systematic reflection, accountable documentation, and openness to critique. Under these

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[4] Julien A. Deonna and Fabrice Teroni, The Emotions: A Philosophical Introduction (New York: Routledge, 2012), esp. Chapter 7.

[5] Jan Slaby, "Affective Intentionality and the Feeling Body," Phenomenology and the Cognitive Sciences 7, no. 4 (2008): 429–444.

[6] The term "artistic research" is often used imprecisely, sometimes as an umbrella for reflective practice or for forms of self-expression that do not generate analyzable insight.

conditions, artistic research can engage embodied emotional processes without reducing them to free-form introspection or anecdote, because it treats concrete artistic decisions—such as tempo, harmonic density, repetition, silence, and relational attunement—as variables that can be described, compared, and refined across cases.

This approach does not reject empirical accountability. On the contrary, it demands a different form of rigour. Instead of relying exclusively on generalised outcomes, it emphasises careful documentation of situated practice: what was done, how it was done, what shifted, and what did not. Small-scale observations—changes in breathing, posture, gaze, duration of engagement, or willingness to remain—become epistemically significant not because they establish causal mechanisms, but because they disclose patterns of emotional orientation that can be examined, compared, and refined.

Crucially, this form of inquiry also treats failure as data. Moments in which music does not hold, fear remains uncontained, or presence cannot be sustained are not anomalies to be excluded; they are essential to understanding the limits of the practice. Artistic research does not promise mastery over existential suffering. It offers attentiveness: a disciplined way of learning from what happens when sound meets vulnerability.

By framing music therapy through emotional epistemology and artistic research, the goal is not to replace medical or psychological models, but to complement them where they fall silent. What is produced is not universal law, but transferable insight: a clearer articulation of how certain practices make it possible, sometimes briefly, sometimes precariously, for patients to remain in relation to their lives when explanation alone cannot help them stay.

6. From Orientation to Practice: Designing the Inquiry and Translating It Clinically

6a. Designing the Inquiry: From Lived Moments to Analyzable Practice

If emotional epistemology concerns how reality is disclosed and navigated through embodied affect, then research in this domain must attend to moments where orientation shifts are visible, however briefly. The aim is not to capture total experiences or to generalise about existential suffering, but to document *micro-transformations* in the organisation of time, body, and relation during musical encounters.

To do this, the unit of analysis is not the patient as a whole, nor the session as a generalised intervention, but the *situated musical moment*. Each session can be understood as a configuration of elements that are actively varied in practice, even when not explicitly named. These include, among others: tempo and pulse, harmonic density, repetition versus development, volume and silence, improvisation versus familiarity, and the relational positioning of therapist and patient.

In this case, a minimal “session micro-log” could serve as one possible research instrument. Rather than adding a bureaucratic burden, it functions as a reflective and analytic scaffold, capturing:

- the musical choices made (what was played, how, and why);

- the immediate embodied and relational shifts observed (breathing, posture, gaze, duration of engagement);
- the temporal trajectory of the session (moments of holding, rupture, return);
- and points of failure or non-response.

Any systematic use of such documentation for research purposes would require appropriate ethics review, informed consent or a justified waiver where applicable, and robust data minimisation and anonymisation procedures in line with institutional policy and data protection law. The present essay proposes a conceptual and methodological frame rather than reporting a study.

Importantly, this documentation does not claim causal proof. Its epistemic value lies in pattern recognition across cases: identifying recurrent configurations in which music reorganises presence, as well as conditions under which it does not. Failure is not noise but a signal, delineating the limits of musical containment.

In this way, artistic research produces knowledge not by abstracting away from practice, but by refining it. Over time, the inquiry clarifies an "intervention anatomy": a more precise understanding of which musical parameters matter, how they interact, and how they modulate emotional orientation in existentially charged clinical contexts.

6b. Translating the Insight: What This Changes for Clinical Practice

From a clinical perspective, none of the above requires adopting a new epistemology or philosophical framework. Its relevance becomes apparent at the level of care itself. Oncology and palliative teams already work daily with phenomena that are not reducible to explanation: uncertainty, fear, anticipatory grief, loss of temporal horizon. The contribution of music therapy, understood through emotional epistemology, is not to resolve these conditions but to create moments in which they can be borne without collapse.

Over the years, I have encountered many patients who ask for emotionally charged, biographical songs—pieces that reliably move them to tears—and yet they want to hear them again and again. At first glance, this can look paradoxical: why deliberately return to music that intensifies sadness? But in practice, it often does not function as a search for "more grief." Rather, the music seems to provide a form for grief—an organised space in which painful emotion can be approached, felt, and held without overwhelming the person. Some patients anecdotally describe feeling calmer or less overwhelmed after such sessions, though responses vary, and these observations are not presented here as research findings. What appears, from the outside, as "sad music making us sad" can therefore be better understood as "sad music making sadness livable."

Translated into clinical terms, the claim is modest and precise: certain musical practices may help stabilise attention, modulate arousal, and sustain relational presence in moments when verbal reassurance and informational clarity are insufficient. This does not replace psychological or medical interventions; it complements them by addressing dimensions of suffering that are enacted bodily and temporally rather than discursively.

For interdisciplinary teams, this reframing shifts the evaluation of music therapy away from vague notions of "support" or "comfort" and toward observable clinical functions. Questions

such as “Does it reduce anxiety?” can be supplemented with more precise ones: Does it help the patient remain present during difficult conversations? Does it prevent emotional escalation? Does it create conditions in which treatment decisions or family encounters become tolerable?

Seen this way, music therapy is neither a distraction nor a decorative addition. It is a practice that intervenes at the level of emotional orientation, enabling patients to remain engaged in their lives despite disruptions to continuity. The epistemic contribution of artistic research lies in articulating this function clearly enough to be recognised, critiqued, and integrated into clinical care—without requiring clinicians to adopt its philosophical vocabulary.

7. Conclusions

This essay began from a simple clinical observation: that the question “Why me?” persists even when explanations are offered, and that its persistence signals not a failure of information, but a rupture in how a person can remain present to their life. In my clinical experience in oncology and palliative care, existential suffering often appears not to be resolved by meaning, reassurance, or stoic declarations of acceptance. It arises from finitude itself—from the sudden exposure of life as fragile, contingent, and irreversible.

Against this background, music therapy emerged not as a solution to existential suffering, but as a practice that can alter the conditions under which such suffering is lived. Music does not answer the question “Why me?” nor does it reconcile the patient with mortality. What it can do, at times, is reorganise presence: restoring breath, attention, temporal continuity, and relational contact when discursive forms of care fall silent. This reorganisation is not propositional knowledge, but a form of knowing enacted through participation—a knowing-with rather than a knowing-about.

By situating these clinical observations within an emotional-epistemological framework, the essay has argued that emotions are not merely subjective states, but embodied ways of navigating reality. In moments of existential crisis, emotions shape what can be endured, what can be approached, and whether one can remain at all. Music therapy, understood in this light, intervenes not at the level of explanation or interpretation, but at the level of emotional orientation.

Artistic research may offer one methodological framework for describing such interventions without reducing them to outcomes alone. By attending to concrete musical decisions and their embodied effects, it becomes possible to articulate how music reorganises experience, when it fails to do so, and under what conditions its use is clinically meaningful. This does not replace medical or psychological models of care; it complements them by addressing dimensions of suffering that are experienced in bodily and temporal terms rather than in discursive terms.

What is ultimately at stake is not the validation of music therapy as an art, nor the elevation of artistic research as an alternative science. It is the recognition that certain forms of suffering demand forms of knowing that do not explain, but sustain; that do not resolve, but hold. In this sense, music therapy occupies a modest but vital place in clinical care: not as an answer to finitude, but as a way of remaining human in its presence.

Disclaimer

This essay presents the author's personal philosophical reflections and theoretical perspectives on clinical music therapy in oncology, palliative, and hospice contexts. It is shared for general information and education only and does not constitute medical, psychological, or legal advice, nor does it establish a therapist–patient relationship. No patient-identifying information is intended or, to the author's knowledge, included. Clinical material is presented in composite and deliberately altered form to minimise re-identification risk and to preserve privacy and confidentiality. The piece does not report research findings and makes no claims of clinical efficacy or survival benefit, as individual circumstances vary and readers should consult qualified healthcare professionals for diagnosis, treatment, or care decisions. The views expressed are solely those of the author. Institutional affiliation is listed for identification only and does not imply institutional review, endorsement, or approval.